

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding reasoning paths :

- 1 . The child exhibits behaviors such as not listening to instructions , difficulty focusing , excessive activity , frequent interruptions , and forgetfulness , which are consistent with attention deficit/hyperactivity disorder (ADHD).
- 2 . The child's history of disruptive behavior , including hitting teachers and classmates , suggests problems with social behavior and impulsivity , which are hallmark features of ADHD .
- 3 . The mother's history of major depressive disorder and the father's Graves ' disease may not directly influence the child's behavior but could indicate a need for comprehensive family consideration in treatment planning .
- 4 . Methimazole is a medication used for the treatment of hyperthyroidism and does not have therapeutic effects on ADHD or other behavioral disorders .
- 5 . Fluoxetine is a selective serotonin reuptake inhibitor (SSRI) used for depression , obsessive-compulsive disorder , and anxiety disorders , but it is not typically first-line treatment for ADHD .

Reasoning Process :

- 1 . **Behavioral Symptoms and ADHD**: The child's symptoms of not listening , difficulty concentrating , excessive activity , interrupting others , and frequent forgetfulness strongly suggest a diagnosis of ADHD . ADHD is characterized by symptoms of inattention , hyperactivity , and impulsivity .
- 2 . **Behavioral Analysis**: The child's behavior of hitting teachers and classmates is consistent with disruptive behavior disorder (DBD), which often co - occurs with ADHD . This suggests a need for intervention targeting both ADHD and DBD .
- 3 . **Family History Consideration**: While the mother's major depressive disorder and the father's Graves ' disease are unrelated to the child's symptoms , they highlight the importance of a holistic approach to treatment , considering family dynamics and potential influence on child behavior .
- 4 . **Treatment Options for ADHD**: Treatment for ADHD typically includes behavioral therapy , stimulant medications (e.g ., methylphenidate or amphetamine), and sometimes non-stimulant medications . SSRI medications like fluoxetine are not typically used for ADHD as they target different psychiatric conditions , such as depression and anxiety .
- 5 . **Role of Methimazole**: Methimazole is used for hyperthyroidism and does not address symptoms of ADHD . Therefore , it is not an appropriate treatment for this child's condition .

Conclusion :

The most appropriate next step in treatment would involve a comprehensive behavioral therapy approach to address both ADHD symptoms and disruptive behavior disorder . Medications like methylphenidate or non-stimulants , along with behavioral interventions , would be considered . Methimazole is not appropriate for treating ADHD and should not be prescribed in this case .

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Fluoxetine

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